
Chronic Pain — The Long Game

When pain has been with you for months or years
— the honest map for managing it well

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HEALING HANDS BY NATE

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Inner Circle Deep Cut — #10

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Introduction

AN INNER CIRCLE DEEP CUT. FOR PEOPLE LIVING with persistent pain who need more than “stretch and rest” advice.

After 12+ years on the table, chronic pain is one of the situations I work with most carefully and most often. Acute pain — the kind you get from a specific injury — is relatively straightforward. It resolves. Chronic pain — pain that’s been present for months or years, often without clear cause — is different. It needs a different framework.

Most of what culture teaches about pain doesn’t apply to chronic pain. “Just push through it” — doesn’t work. “Take some ibuprofen and rest” — temporarily helps, doesn’t address the underlying issue. “It’ll go away on its own” — sometimes, but often won’t. The advice that works for acute pain often fails or makes chronic pain worse.

This deep cut is for people living with chronic pain. The honest framework. What it is, what it isn’t, what helps, what doesn’t, when to escalate, and how bodywork fits into the bigger picture.

Critical up front: This is educational, focused on what bodywork can support and what a thoughtful self-management practice looks like. Chronic pain often requires a medical team — physician, pain specialist, sometimes physical therapist, sometimes mental health support. This document doesn't substitute for any of that. It supplements it.

CHAPTER 1

What Chronic Pain Actually Is

THE FIRST THING TO UNDERSTAND: CHRONIC pain is fundamentally different from acute pain.

Acute pain

Acute pain is the body's alarm system working correctly. You injure tissue, nerves send signals to the brain, the brain interprets the signals as pain, you protect the area, it heals, the pain goes away.

This process is well-designed and adaptive. The pain protects you while the tissue heals.

Chronic pain

Chronic pain (defined as pain persisting beyond normal tissue healing time — usually 3+ months) is more complex. The original tissue damage may have healed entirely, but pain continues. Sometimes pain emerges without any identifiable original injury.

What's happening in chronic pain:

Nervous system sensitization. The nerves that originally signaled tissue damage become hypersensit-

ive. They signal pain at lower thresholds. Signals that wouldn't normally register as painful become painful.

Central nervous system changes. The brain regions involved in pain processing become more active and more easily triggered. Pain pathways become “well-worn” — easier to activate over time.

Reduced inhibition of pain signals. The normal mechanisms that dampen pain signals become less effective.

Emotional and contextual factors. Stress, sleep, mood, social context all influence pain perception. In chronic pain, these factors play a larger role than in acute pain.

This isn't “psychological” pain — the pain is absolutely real. But the pain isn't a direct readout of tissue damage anymore. It's a more complex experience involving the nervous system, the brain, and the broader life context.

Why this matters

The implications change everything:

- “Find the tissue damage and fix it” often doesn't work because the pain isn't proportional to tissue damage anymore
- Imaging (MRI, X-ray) often shows minor findings that may or may not be related to the pain
- Treatments aimed solely at tissue may help temporarily but rarely resolve chronic pain

- Approaches that address the nervous system, stress, sleep, and life context often help more than tissue-focused approaches alone
- Recovery from chronic pain looks different than recovery from acute pain — it's gradual, non-linear, and rarely "complete cure" pattern

This is the modern pain science consensus. It contradicts a lot of intuitive thinking about pain. It's also the framework that actually helps people who've been stuck in chronic pain for years.

CHAPTER 2

The Biopsychosocial Model

MODERN PAIN MANAGEMENT USES WHAT'S called the biopsychosocial model. Chronic pain involves three intersecting factors:

Biological factors

- Tissue health (current state, not just original injury)
- Nervous system sensitization
- Inflammation
- Posture and biomechanics
- Sleep quality
- Hormonal balance
- Genetics

Psychological factors

- Stress
- Anxiety and depression
- Pain beliefs (catastrophizing vs. coping)
- Past trauma
- Mood

- Attention patterns toward pain

Social factors

- Work demands and environment
- Family relationships
- Social support (or isolation)
- Financial pressure
- Cultural beliefs about pain
- Access to healthcare

All three factors influence pain experience. Effective chronic pain management addresses all three. Approaches that focus on only one (usually the biological) often produce limited or temporary results.

This isn't about minimizing pain. It's about understanding what affects it so you can address it effectively.

CHAPTER 3

Patterns I See Most Often

Pattern 1 — “I’ve been to every specialist and nothing works”

COMMON STORY. YEARS OF SEEKING DIAGNOSIS and treatment. Multiple specialists. Multiple opinions. Many treatments tried. Some helped briefly, none lasted. Frustration and exhaustion accumulating along with the pain.

What’s often happening: The person has been working within a model that assumed tissue damage was the issue. Treatments focused on tissue. Underlying nervous system sensitization, stress patterns, and life context factors weren’t adequately addressed.

What can help: Shifting framework to bio-psychosocial approach. Often involves working with a pain specialist or pain psychologist who understands modern pain science. Bodywork can be part of the multi-modal approach.

Pattern 2 — “My imaging shows damage, so this pain makes sense”

Common assumption. MRI shows disc bulge, arthritis, rotator cuff tear, etc. Person attributes pain entirely to the imaging finding.

What’s often happening: Imaging findings poorly correlate with pain in many cases. Large studies have shown that asymptomatic people (no pain) often have the same imaging findings as people with significant pain. The imaging shows what’s structurally present, not what’s causing pain.

What can help: Understanding that imaging findings aren’t always the pain source. Working with practitioners who understand this. Sometimes the imaging finding is the cause; sometimes it’s incidental. Treatment approach should be informed by symptoms and response to interventions, not solely by imaging.

Pattern 3 — “I just need to push through and stay active”

Some people try to power through chronic pain by maintaining high activity levels. Sometimes this works. Often it backfires — periods of overdoing it followed by crashes, with overall function declining over time.

What’s often happening: The boom-bust cycle. Push through pain → trigger flare → crash → reduced

activity → guilt → push again → flare again. Net effect is reduced overall function.

What can help: Pacing — sustainable activity levels with planned rest. Building activity tolerance gradually rather than testing limits constantly. Working with PT or movement specialist on graded loading.

Pattern 4 — “I’ve stopped doing everything because everything hurts”

Opposite pattern. Person withdraws from activities to avoid pain. Over time, withdrawal compounds the problem — deconditioning, social isolation, depression, and increased pain sensitivity.

What’s often happening: Fear-avoidance pattern. Reasonable initial response to pain (avoid things that hurt) becomes maladaptive over time as it shrinks life.

What can help: Graded exposure — gradually expanding activity within tolerance. Often benefits from working with PT or pain psychologist trained in graded exposure approaches. Sometimes counterintuitive interventions (gentle movement when avoidance is the pattern) help most.

Pattern 5 — Trauma-related chronic pain

Some chronic pain patterns are tied to past trauma. The body holds the imprint of unresolved trauma. Pain

patterns often persist beyond what tissue damage would explain.

What's often happening: The nervous system remains in chronic activation related to trauma exposure. Body areas tied to specific trauma may hold sensitivity. Standard tissue-focused approaches may help temporarily but don't address the underlying trauma layer.

What can help: Trauma-informed bodywork (some practitioners specialize in this). Trauma-specific therapy (EMDR, somatic experiencing). Combination often produces shifts that neither alone would.

Pattern 6 — Manageable chronic pain

This is the goal pattern. Person has chronic pain but it doesn't dominate their life. They have practices that help. They know their patterns. They flare sometimes but recover. They maintain a meaningful life despite the pain.

What's often happening: Multi-modal management approach over time. Acceptance of chronic pain as part of life context (not victory or defeat). Sustainable practices that produce more good days than bad.

What can help: Continuing what's working. Periodic reassessment as life circumstances change. Maintaining the medical/professional support network.

CHAPTER 4

Self-Assessment for Chronic Pain

IF YOU HAVE PAIN THAT'S BEEN PRESENT FOR 3+ months, this assessment helps map where you are.

Pain characteristics

Duration: _ months/years **Constant vs intermittent:** _____ **Intensity range (0-10):** Best ___ / Worst ___ / **Average** ___ **Primary location(s):** _____ **Specific original injury or trigger:** _____

What helps and hurts

Things that consistently reduce pain: - Heat - Cold - Movement - Rest - Specific positions - Specific exercises - Bodywork - Medication - Other: _____

Things that consistently increase pain: - Specific activities - Specific positions - Stress - Poor sleep - Weather - Other: _____

Impact on function

Sleep impacted: [] No / [] Some / [] Significant / [] Severe **Work impacted:** [] No / [] Some / [] Significant

ant / ☐ Severe **Relationships impacted:** ☐ No / ☐
Some / ☐ Significant / ☐ Severe **Activities you've
stopped due to pain:** _____

Mood and stress

Mood (1 = depressed, 10 = thriving): ☐ **Stress
level (1 = calm, 10 = constant high stress):** ☐
Anxiety about pain (1 = none, 10 = constant): ☐
Sleep quality (1 = terrible, 10 = excellent): ☐

Treatment history

Have you tried (mark all): - ☐ Primary care physi-
cian - ☐ Specialist consultation (which:) - ☐ **Phys-
ical therapy** - ☐ **Bodywork / massage therapy** - ☐
Chiropractic - ☐ **Acupuncture** - ☐ **Pain
medication (which:)** - ☐ Pain specialist - ☐ Pain
psychologist - ☐ Surgery - ☐ Injection therapies - ☐
Mental health therapy - ☐ Mindfulness / meditation -
☐ Diet changes - ☐ Exercise programs

Honest reflection

Pattern I most identify with from the list above:

**What feels most missing from my current ap-
proach:** _____

CHAPTER 5

What Actually Helps Chronic Pain

MODERN PAIN MANAGEMENT RESEARCH CONSISTENTLY points to multi-modal approaches that address multiple factors simultaneously. The interventions with the strongest research support:

1. Movement and gradual activity

Counterintuitive but well-supported: people with chronic pain typically improve with appropriate, graded movement. Not pushing through pain — gradual expansion of what the body tolerates.

What this looks like in practice: - Daily walking, even short walks - Gentle mobility work (the modules in this membership) - Gradual building of activity tolerance - Working with PT for specific guided progression

2. Sleep optimization

Poor sleep amplifies pain. Good sleep dampens it. Sleep is one of the most leveraged interventions for chronic pain.

See IC8 (Sleep As Foundation) for the deep dive.

3. Stress management and nervous system work

Chronic stress and sympathetic dominance amplify chronic pain. Practices that down-regulate the nervous system reduce pain intensity over time.

See IC4 (Nervous System Self-Assessment), Module 2 (Breath as Regulation), Module 4 (The Vagus Reset).

4. Bodywork

Targeted bodywork can: - Release tissue patterns that are contributing - Down-regulate nervous system through touch - Provide pain relief windows for movement - Support overall recovery context

For chronic pain specifically, the approach is different than for acute pain. Generally gentler, more focused on nervous system regulation, more patient with the timeline.

5. Pain education

Just understanding modern pain science often reduces pain. Research on “pain neuroscience education” consistently shows that learning how pain works reduces pain perception and disability.

This document is part of that education.

6. Mental health support

For pain that has psychological components (most chronic pain has some), mental health support is genuinely effective. Cognitive behavioral therapy for chronic pain (CBT-CP) has the strongest research support. Acceptance and commitment therapy (ACT) also shows good results.

7. Mindfulness practices

Mindfulness meditation has substantial research support for chronic pain management. Not eliminating pain — changing relationship to pain in ways that reduce suffering.

8. Social support

Isolation worsens chronic pain. Maintaining social connections, support groups (in-person or online), connection with people who understand chronic pain — all help.

9. Pacing strategies

Learning to do enough but not too much. Sustainable activity levels with planned rest. Avoiding boom-bust cycles.

10. Sometimes medication

Pain medication has a place in chronic pain management. Especially: - During flares - For pain that pre-

vents sleep - As bridge during periods of significant disability - For specific pain types (neuropathic pain often responds to specific medications)

Best managed by a pain specialist who can navigate the complex risk-benefit tradeoffs over time.

CHAPTER 6

What Often Doesn't Help (or Helps Less Than Expected)

SOME INTERVENTIONS ARE COMMONLY TRIED BUT have limited evidence for chronic pain specifically:

Surgery for non-acute issues

Surgery for back pain (in absence of red flag symptoms) consistently underperforms expectations. Multiple studies show surgical and non-surgical outcomes are similar for many common chronic back pain situations. Surgery has real risks. Not “never surgery” — but the bar for surgery should be high.

Opioids for chronic non-cancer pain

The evidence for long-term opioid use in chronic non-cancer pain is increasingly poor. Tolerance develops. Side effects accumulate. Dependence risk is real. Opioids have a place for acute severe pain and end-of-life pain — limited role for years-long chronic pain management.

Aggressive stretching of “tight” areas

Repeatedly aggressively stretching areas that feel “tight” in chronic pain can actually sensitize them further. Sometimes the perception of tightness reflects nervous system sensitivity, not actual tissue restriction.

Single-modality approaches

“Just chiropractic” or “just PT” or “just massage” or “just yoga” — single modalities for chronic pain often disappoint. Multi-modal approaches consistently outperform.

Most supplements

Some supplements help some people. Most don't deliver what's marketed. Magnesium and omega-3 have some research support. Most “joint” and “pain” supplements have weak evidence.

Passive treatments alone

Anything you receive passively (massage, adjustments, injections, etc.) without active participation typically has limited lasting effect for chronic pain. The combination of passive treatment + active practices is much more effective than either alone.

CHAPTER 7

How Bodywork Specifically Fits Chronic Pain

TO BE HONEST ABOUT MY OWN SCOPE:

What I can do for chronic pain

- Provide tissue release and pain relief windows
- Support nervous system regulation through touch
- Help identify and address physical patterns contributing to pain
- Be part of a multi-modal team
- Provide consistent, predictable maintenance support
- Educate about body mechanics and self-care

What I can't do

- Diagnose pain conditions
- “Cure” chronic pain in any complete sense (though I can support significant reduction in many cases)
- Prescribe medication

- Provide pain psychology or mental health treatment
- Replace medical management

How to use bodywork in chronic pain

Consistent maintenance: Regular sessions (often every 3-4 weeks) typically work better than sporadic intense interventions for chronic pain.

Combined with active practices: What you do between sessions matters as much as the sessions themselves.

Realistic expectations: Chronic pain rarely resolves completely with bodywork alone. Significant improvement and better management are realistic; complete resolution sometimes isn't.

Communication: Tell me what's happening between sessions. Tell me what helps and hurts. The work is more effective with that information.

CHAPTER 8

Building Your Long-Game Plan

CHRONIC PAIN MANAGEMENT IS A LONG GAME. Here's the framework for a sustainable approach.

Step 1 — Assemble your team

Most chronic pain benefits from a multi-disciplinary team:

- **Primary care physician:** Quarterback. Coordinates referrals, monitors overall health.
- **Pain specialist:** If pain is significant, ongoing, or unresponsive to primary care.
- **Physical therapist:** For graded loading, movement retraining, specific exercises.
- **Bodyworker (me):** For tissue and nervous-system work, consistent maintenance.
- **Mental health professional:** For pain-related anxiety, depression, or stress management.
- **Possibly: pain psychologist, sleep specialist, registered dietitian** depending on contributing factors.

You may not need all of these. Most people with significant chronic pain benefit from 3-4 of them.

Step 2 — Build your daily practice

Daily, non-negotiable basics: - Sleep hygiene (per IC8 and Module 3) - Movement practice (gentle daily mobility per Module 8 and IC9) - Stress management (per Modules 1, 2, 4 and IC4) - Nutrition basics (working with a dietitian if helpful)

Step 3 — Track patterns

Use the workbook to track pain patterns, flares, what helps, what doesn't. Over time, this data is your most useful asset for understanding YOUR chronic pain specifically.

Step 4 — Plan for flares

Chronic pain typically includes flares. Having a flare plan reduces the catastrophizing that worsens flares:

- What do I do when pain spikes?
- What activities do I scale back?
- What practices do I increase?
- Who do I contact?
- How do I know if it warrants emergency care?

Step 5 — Accept the long timeline

Chronic pain that's been present for years rarely resolves in weeks or months. Improvement typically hap-

pens gradually over 6-12 months with sustained multi-modal effort. Some pain may never fully resolve but can become much more manageable.

This timeline isn't a failure — it's just the reality of working with sensitized nervous systems and accumulated patterns.

CHAPTER 9

When to Escalate Care

ALWAYS ESCALATE (SOMETIMES URGENTLY) FOR:

- New, severe, or different pain
- Pain accompanied by neurological symptoms (numbness, tingling, weakness)
- Bowel or bladder dysfunction
- Unexplained weight loss
- Fever with pain
- Pain that wakes you regularly from sleep
- Significant change in pain pattern
- Worsening despite consistent treatment
- Thoughts of self-harm or suicide related to pain — contact 988 or your physician immediately

Chronic pain is exhausting. It increases risk for depression and suicidal ideation. Don't navigate severe pain alone. Build a support team.

CHAPTER 10

What to Expect Over a Year of Consistent Multi-Modal Work

MONTH 1-2: ASSEMBLING SUPPORT TEAM. Establishing daily practices. Initial setbacks as practices stabilize.

Month 3-4: Daily practices become routine. Some early gains. Identifying personal pattern triggers.

Month 5-6: Often a notable plateau. Don't quit here. The cumulative effects haven't yet shown up.

Month 7-9: Cumulative gains often start showing. More good days than bad. Better flare recovery.

Month 10-12: Significant overall improvement in most cases. Pain may persist but quality of life much better. Better management tools. More confidence.

This trajectory assumes: - Consistent multi-modal approach - Realistic expectations (improvement, not cure) - Working with appropriate medical/professional support - Sticking with the basics (sleep, movement, stress management) even when they feel inadequate

CHAPTER 11

Closing

CHRONIC PAIN IS REAL. IT'S ALSO MORE RESPONSIVE to systematic management than most people in chronic pain currently believe.

The framework matters. Acute pain rules don't apply. Multi-modal approaches outperform single interventions. Daily practices matter as much as professional treatments. Mental health components are real and addressable. The timeline is longer than anyone wants but the gains are real.

This document is a starting framework. Real chronic pain work happens over months and years, with a team, with patience, and with the understanding that "managed well" is often more realistic than "completely cured."

If you're in chronic pain right now — start with sleep, start with daily movement, start with assembling your team. The compound effects of consistent practice over time produce changes that single dramatic interventions rarely match.

The companion workbook has a chronic pain self-assessment, a 90-day pain tracker, a flare response plan, and three reflection prompts to help you build your long-game approach.

— Nate



This book was crafted by Nate Ratcliff,
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